

Provider Net-Cost Recovery Scenario Summary					
Header		Value		Header	Value
Product	Adrabetadex		Sourcing model	Buy-and-bill	
Scenario name	Default 340B COE HOPD, Pass-Through		Pass-through status	Active	
Date generated	4/23/2026		Reimbursement basis	WAC %	
Site type	HOPD				
Payer type	Medicare				
340B status	Yes				
Coding status	Pass-through active				
Selected Scenario Summary	Value		Per-Dose Economics	Value	
Site of care	HOPD	Acquisition	Drug acquisition per dose	\$29,991.00	
Payer type	Medicare	Gross drug	Allowed drug	\$40,170.00	
340B status	Yes		Drug spread per dose	\$10,179.00	
Coding status	Pass-through active		Procedure revenue per	\$500.00	
Pass-through status	Active		Non-drug operating cost	\$1,700.00	
Sourcing model	Buy-and-bill		Denial drag per dose	\$950.73	
Reimbursement basis	WAC %		Financing drag per dose	\$493.00	
			Net operating margin per	\$7,535.27	
			Break-even support per	\$0.00	
Annual Economics	Value		Risk Flags	Result	Explanation
Annual administrations	26		Margin risk	Low margin risk	Per-dose margin has stronger cushion.
Annual drug acquisition	\$779,766.00		Cash exposure risk	Moderate cash exposure	Monitor outstanding administrations and
Annual allowed drug reimbursement	\$1,044,420.00		Operational risk	Lower operational risk	Operational assumptions are relatively stable.
Annual non-drug operating cost	\$44,200.00				
Net operating margin	\$195,916.95				
Annual break-even support	\$0.00				
Cash exposure	\$179,946.00				
Annual 340B acquisition advantage	\$234,234.00				
Customer Conversation Takeaways					
1. 340B acquisition materially improves provider economics.					
2. Pass-through and permanent HCPCS timing reduce payment uncertainty across the launch bridge.					
3. Clean PA, NDC, billing, wastage, and 340B documentation are required to convert coverage into payment.					
Estimates are for planning purposes only and do not guarantee coverage, coding, payment, or reimbursement. Providers should verify coding, billing, payer rules, contract terms, and documentation requirements before claim submission.					